BASELINE → TREATMENT → FOLLOW-UP

# Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

| Check | Before starting | During treatment | Between visits | Response / follow-up | After a hold or stop |
|---|---|---|---|---|---|
| **Bloods and organ safety** | Blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune conditions, transplants, previous immune reactions and all current medicines. | Blood count, kidney/liver tests and glucose before every 3-week cycle. thyroid-stimulating hormone (TSH) is checked every 3–6 weeks, including between treatment visits when needed. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity. | Contact the team for new symptoms; extra tests may be needed between scheduled checks. | Safety bloods do not by themselves show whether cancer is responding. | Continue to report possible immune effects after the last dose; your team records the follow-up tests and dates. Keep any prescribed steroid taper, hormone replacement and specialist follow-up. Scans follow your individual disease plan. |
| **Cancer, skin and daily function** | Record diagnosis, stage, current disease sites and the baseline assessment. | Keep the source-specific skin and clinical reviews and the individually agreed response assessments. | Record new lesions, lumps, wound changes, pain and effects on usual activities. | Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed. | Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team. |
| **Medicines and continuing precautions** | Review all medicines and the pregnancy/fertility plan. | Review treatment taken or received, holds, adverse effects and supportive medicines. | Keep the actual urgent contacts and do not restart a held medicine yourself. | Bring the medicine list and treatment summary to other services. | Discuss fertility before starting. Use effective contraception during treatment and for at least 4 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself. |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00455 v16: current regimen source

THE WHOLE TREATMENT JOURNEY

# Pembrolizumab · every 3 weeks · melanoma before and after surgery: treatment and checks

**Selected stage IIIB melanoma eligible for the NCCP adjuvant indication: the treating clinician may choose pembrolizumab 200 mg before and after surgery. This is a distinct, discretionary perioperative plan.**

Pembrolizumab 200 mg every 3 weeks: 3 doses before surgery, then 15 doses after surgery, subject to recovery and clinical review. The team sets the operation date and postoperative restart; do not assume that surgery or a held dose follows the routine three-week calendar.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | During treatment | Between visits | Response / follow-up | After a hold or stop |
|---|---|---|---|---|---|
| **Bloods and organ safety** NCCP | Blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune conditions, transplants, previous immune reactions and all current medicines. | Blood count, kidney/liver tests and glucose before every 3-week cycle. TSH is checked every 3–6 weeks, including between treatment visits when needed. Cortisol and other tests are added when clinically indicated. Report new symptoms at every visit and between visits; normal blood results do not rule out immune toxicity. | Contact the team for new symptoms; extra tests may be needed between scheduled checks. | Safety bloods do not by themselves show whether cancer is responding. | Continue to report possible immune effects after the last dose; your team records the follow-up tests and dates. Keep any prescribed steroid taper, hormone replacement and specialist follow-up. Scans follow your individual disease plan. |
| **Cancer, skin and daily function** NCCP | Record diagnosis, stage, current disease sites and the baseline assessment. | Keep the source-specific skin and clinical reviews and the individually agreed response assessments. | Record new lesions, lumps, wound changes, pain and effects on usual activities. | Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed. | Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team. |

| Care / check | Before starting | During treatment | Between visits | Response / follow-up | After a hold or stop |
|---|---|---|---|---|---|
| **Medicines and continuing precautions** NCCP | Review all medicines and the pregnancy/fertility plan. | Review treatment taken or received, holds, adverse effects and supportive medicines. | Keep the actual urgent contacts and do not restart a held medicine yourself. | Bring the medicine list and treatment summary to other services. | Discuss fertility before starting. Use effective contraception during treatment and for at least 4 months after the last dose. Tell the team promptly about a possible pregnancy. Discuss breastfeeding restrictions with the team before treatment; do not breastfeed without their advice. Do not stop prescribed steroids or hormone replacement yourself. |

## What each check is for

### 1 Before surgery

3 planned doses

Pembrolizumab is given before and after planned surgery in this selected melanoma pathway. The team explains the expected benefit, uncertainty, surgical plan and how side effects could change the sequence.

**Ask / act:** Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

### 2 Surgery and recovery

Team-agreed timing

Pembrolizumab is given before and after planned surgery in this selected melanoma pathway. The team explains the expected benefit, uncertainty, surgical plan and how side effects could change the sequence.

**Ask / act:** Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

### 3 After surgery

15 planned doses

Pembrolizumab is given before and after planned surgery in this selected melanoma pathway. The team explains the expected benefit, uncertainty, surgical plan and how side effects could change the sequence.

**Ask / act:** Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.41